A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

30 April 2026

Dr Jane Healy
Glen Iris Medical Group

Dear Dr Healy,

Re: Ms Kate Gengoult-Smith
DOB: 31/03/1976 Phone: 0425 324 173
Address: 1 Boyanda Road, GLEN IRIS, VIC, 3146
Email: kategs@gmail.com
Medicare: 3360266241 / 1

Summary

- **Clinically inactive Crohn's disease with improving faecal calprotectin levels.**
- **Trial of sandomigran and mebeverine for visceral hypersensitivity and suspected abdominal migraine.**
- **Monitoring of infliximab levels and faecal calprotectin in three months.**
- **No action required by you.**

I had the pleasure of conducting a review with Kate. You will be pleased to note that her Crohn's disease remains clinically inactive. Her most recent biochemical markers are encouraging, with a downward trend in her faecal calprotectin , as she had a short lived period of symptoms with a calprotectin >300. The repeat is 69.

We discussed Kate's nutritional status and weight, which have remained stable over the last 18 months. This is a positive development given her clinical history. She has incorporated high-protein supplements into her diet with good effect. I have reinforced the importance of avoiding restrictive dietary habits to ensure she maintains this stability and continues her recovery.

Despite the lack of active inflammation, Kate continues to experience symptoms consistent with visceral hypersensitivity, including bloating and abdominal discomfort. We discussed the potential for an abdominal migraine component to her presentation. While more common in paediatric populations, it remains a possibility in her case. We have therefore agreed to a trial of sandomigran. Additionally, she will trial mebeverine as an antispasmodic to assist with her symptoms of bloating and pain.

Regarding her recent reports of minor perianal bleeding and difficulty with bowel evacuation, I have advised the use of over-the-counter Proctosedyl ointment applied twice daily. Given her recent colonoscopy findings and stable inflammatory markers, I do not believe these symptoms represent a flare of her Crohn's disease.

Impression and Plan

Kate's Crohn's disease is currently in remission, and the primary focus of our management is now the control of her functional gastrointestinal symptoms and the maintenance of her nutritional health. I have requested that she repeat her infliximab levels and faecal calprotectin shortly before our next review. She will proceed

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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with trials of pizotifen and mebeverine, and we will monitor her progress. I have scheduled a follow-up appointment with Kate in three months' time.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Miss Sarah Egan, Miss Sarah Gilliland, Dr Karen Gwee

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